

ST THOMAS' HEALTHCARE NHS FOUNDATION TRUST

Department of Endocrinology & General Internal Medicine

St Thomas' Hospital, Westminster Bridge Road, London SE1 7EH

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Patient Name: Eleanor Jane DAVIES

NHS Number: 492 883 1047

Date of Birth: 14 May 1968 (Age: 58)

Hospital No. (MRN): RTH-882190

Address: 42 Highbury Walk, London, N5 2ER

Date of Consultation: 08 September 2026

Clinic Reference: END-2026-0941A

Referring GP: Dr Alistair Finch (GMC: 4892103)

Practice: Riverside Surgery, London SE1 2BA

Consultant: Dr Simon Thorne, FRCP (GMC: 6184920)

CLINICAL CONSULTATION SUMMARY / CLINIC LETTER

Dear Dr Finch,

Thank you for referring Mrs Eleanor Davies to our Endocrinology and Metabolic Outpatient Clinic for assessment of suboptimal glycemic control and persistent peripheral dysesthesia.

Reason for Referral & Presenting Complaint

Mrs Davies was reviewed today in follow-up of deteriorating glycemic parameters (most recent community HbA1c: 78 mmol/mol) despite dual oral antidiabetic therapy. In addition, she reports a 4-month history of bilateral symmetrical burning sensations and numbness affecting both distal lower limbs (stocking distribution), worse at night.

Medical & Surgical History

- Type 2 Diabetes Mellitus (diagnosed 2017)
- Essential Hypertension (diagnosed 2015)
- Primary Hypercholesterolaemia
- Elective Laparoscopic Cholecystectomy (2021)
- Nil known drug allergies (NKDA)

Current Medications

- Metformin 1,000 mg twice daily (oral)
- Gliclazide 80 mg once daily with breakfast (oral)
- Ramipril 5 mg once daily (oral)
- Atorvastatin 20 mg once daily at night (oral)

Clinical Examination & Observations

- **General:** Alert, oriented $\times 3$, comfortable at rest. BMI: 29.4 kg/m² (Weight: 81.2 kg, Height: 1.66 m).
- **Vital Signs:** BP 138/84 mmHg, HR 72 bpm regular, SpO₂ 98% room air, Temp 36.7°C.

- **Cardiovascular & Respiratory:** Heart sounds I + II present, nil murmurs. Chest clear to auscultation bilaterally.
- **Neurological (Lower Limbs):** Reduced 10g monofilament sensation in bilateral distal lower extremities extending to mid-shins. Absent ankle jerks bilaterally. Vibration sensation mildly diminished over the first metatarsophalangeal joints. No skin ulceration or active foot infection. Peripheral pulses (dorsalis pedis and posterior tibial) palpable.

Recent Investigations (01/09/2026)

Test	Result	Reference Range	Status / Comment
HbA1c	78 mmol/mol	20–42 mmol/mol	Elevated (suboptimal control)
eGFR	74 mL/min/1.73m ²	>60 mL/min/1.73m ²	Stable CKD Stage G2
Serum Creatinine	84 µmol/L	45–90 µmol/L	Normal
Urine Albumin/Creatinine Ratio	4.8 mg/mmol	<3.0 mg/mmol	Mild microalbuminuria (A2)
Total Cholesterol	4.6 mmol/L	<5.0 mmol/L	Target met
Serum Vitamin B12	320 ng/L	180–900 ng/L	Normal
Thyroid Stimulating Hormone	2.1 mIU/L	0.27–4.20 mIU/L	Normal

Clinical Impression

1. **Suboptimally controlled Type 2 Diabetes Mellitus** requiring intensification of therapy.
2. **Early Diabetic Sensorimotor Polyneuropathy**, clinically evident on sensory examination.
3. **Diabetic Nephropathy (Stage A2 Microalbuminuria)** with preserved glomerular filtration.

Management Plan & Recommendations

1. **Pharmacotherapy Adjustment:**
 - Discontinue Gliclazide to minimize hypoglycemia risk.
 - Initiate Empagliflozin (SGLT2 inhibitor) 10 mg once daily, providing both glycaemic and renal protection per NICE NG28 guidelines.
 - Continue Metformin 1,000 mg twice daily.
 - Titrate Ramipril from 5 mg to 10 mg once daily to address microalbuminuria, subject to repeat U&Es in 2 weeks.
 - Prescribe Duloxetine 30 mg once daily for 2 weeks, increasing to 60 mg once daily for symptomatic relief of painful diabetic neuropathy.
2. **Referrals:** Direct referral placed to the Community Diabetic Podiatry Team for annual foot care plan and protective footwear assessment.
3. **Patient Education:** Counseled Mrs Davies regarding sick-day rules for SGLT2 inhibitors and monitoring for symptoms of euglycaemic DKA and mycotic genital infections.
4. **Follow-up:** Review in Endocrinology Outpatient Clinic in 4 months with repeat HbA1c, renal profile, and UACR prior to the appointment.

Yours sincerely,

Dr Simon Thorne, MA, FRCP

Consultant Physician in Endocrinology & Diabetes

GMC Registration No: 6184920

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